Cardiology Follow-up and 2D Echocardiogram

Cardiology Review Note

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Facility: Metro Heart & Vascular Institute, Outpatient Clinic

Date of Consultation: 15-Sep-2026 (Recent review, 2 years post-PCI)

Attending Cardiologist: Dr. Arvind Swaminathan, MD, DM (Cardiology), FESC

Patient Name: Devendra Varma | Age: 56 | Gender: Male

Clinical Progress and Physical Findings

Functional Capacity: Patient reports persistent NYHA Class II exertional breathlessness on climbing 1 flight of stairs or walking fast on level ground. No recurrent angina at rest.

Vitals: BP 148/92 mmHg (Uncontrolled hypertension despite anti-hypertensives). Pulse 78 bpm regular.

Cardiovascular Exam: Apex beat shifted to 6th intercostal space in anterior axillary line (cardiomegaly). Soft S3 gallop audible. Grade 2/6 holosystolic murmur at apex.

Recent 2D Echocardiography (15-Sep-2026): Left Ventricular Ejection Fraction = 44%. Persistent regional wall motion abnormality in LAD territory. Left ventricular internal diameter in diastole (LVIDd) 56 mm (mild dilatation). Dilated left atrium (LAVI 36 mL/m²). Mild-to-moderate ischemic mitral regurgitation.

Cardiologist Assessment: Chronic ischemic cardiomyopathy secondary to past anterior STEMI. Residual moderate systolic dysfunction. Hypertension suboptimal control. At high risk of heart failure decompensation. Needs continuation of cardioprotective regimen.

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